



PARACETAMOL - 120 (BANANA) SYRUP

COMPOSITION:

Each 5 ml contains
Paracetamol

120mg

Methylparaben 0.1% w/v as preservative.

PRESENTATION:

Yellow colored banana flavoured suspension.

INDICATIONS:

For the relief of fever, symptoms of colds and influenza, teething pains and headache.

PHARMACOLOGY:

Mechanism of Action:

Paracetamol is a para-aminophenol derivative with analgesic and antipyretic effects. Paracetamol relieves pain of moderate intensity, such as unusually occurs in headache and dysmenorrhea, and in many muscle, joint, and peripheral nerve disorders. It reduces fever by inhibiting the action of endogenous pyrogen on the hypothalamic heat-regulation centers. The locus of the analgesic effect is uncertain. The mechanism of the antipyretic and analgesic effect is also uncertain.

Pharmacokinetics:

Paracetamol is readily absorbed from the gastrointestinal tract with peak plasma concentrations occurring about 10 to 60 minutes after oral administration. The plasma half-time is 1-3 hours. It is distributed in most body tissues and it crosses the placenta and is present in breast milk. Paracetamol is metabolised predominantly in the liver and excreted in the urine mainly as glucuronide and sulphate conjugates. Less than 5% is excreted as unchanged paracetamol.

DOSAGE AND ADMINISTRATION:

For oral administration only.

SHAKE WELL BEFORE USE.

Children 3 months to 1 year old :

60mg - 120mg (2.5ml - 5 ml)

1 - 6 years : 120mg - 240mg (5ml - 10ml)

6 - 12 years : 240mg - 480mg (10ml - 20ml)

to be taken 3 or 4 times a day.

CONTRAINDICATION:

Contraindicated in patient with nephropathy.

PRECAUTION:

It should be given with care to patients with impaired kidney or liver functions. Chronic use should be avoided. Paracetamol can induce synthesis of the hepatic microsomal enzymes. The prothrombinopenic effect of the oral anticoagulants may be increased by chronic administration of full doses of paracetamol.

This preparation contains PARACETAMOL. Do not take any other PARACETAMOL containing medicines at the same time.

SIDE EFFECTS:

Skin rash and other allergic reactions occur occasionally. The rash is usually erythematous or urticarial but sometimes more serious and may be accompanied by drug fever and mucosal lesions. Patients allergic to the salicylates do not exhibit cross-sensitivity to paracetamol. In a few cases, the use of paracetamol has been associated with the occurrence of neutropenia, pancytopenia, and leukopenia.

OVERDOSAGE AND TREATMENT:

Symptoms of overdosage in accidental ingestion of large doses of the cream may cause nausea, vomiting and abdominal pain. Liver damage may be apparent after 12 hours and may lead to hepatic failure, encephalopathy and death. Other severe effects include metabolic acidosis, renal failure, and cardiac arrhythmias. Chronic overdosage led to T-cell defects.

In non-fatal cases, the peptic lesions are reversible over a period of weeks or months. Measurement of the plasma half-time of paracetamol during the first day of acute poisoning provides an early indication of the severity of the liver injury. Hepatic necrosis should be anticipated if the half-time exceeds 4 hours, and hepatic coma is likely if the half-time is greater than 12 hours. Encephalopathy should also be anticipated if serum bilirubin concentration exceeds 4mg / 100ml during the first 5 days.

Treatment of acute paracetamol overdosage is purely symptomatic, vigorous supportive therapy is essential in severe intoxication. Induction of vomiting or gastric lavage should be performed in all cases, follow by oral advocated for patients with a plasma paracetamol concentration greater than 120mg / ml 4 hours after drug ingestion. Peritoneal dialysis with albumin in the dialysate may be useful. Cysteamine or n-acetyl-cysteine intravenously, or 1-methionine orally appear to protect the liver if administered within 10 hours of ingestion and overdosage. There are no known antidotes for paracetamol.

DRUG INTERACTIONS :

The prothrombinopenic effect of the oral anticoagulants may be increased by chronic administration of full doses of paracetamol.

TOXICOLOGY:

In recommended therapeutic dosage, paracetamol is usually well-tolerated. The most serious adverse effect of acute overdosage is dose-dependent, potentially fatal hepatic necrosis. Renal tubular, necrosis and hypoglycemic coma may also occur. In adults, hepatotoxicity may occur after ingestion of a single dose of 10-15kg or more is potentially fatal. The hepatotoxicity may progress to encephalopathy, coma and death. Transient azotemia is apparent in most patients and acute renal failure occurs in some. Antihistamines increase paracetamol toxicity in animals. Marked methaemoglobinaemia, suphaemoglobin formation, cyanosis and functional anemia were prominent characteristics of poisoning by acetanilid, the parent member of the aminophenol group.

STORAGE:

Keep container well closed. Store below 30°C . Protect from light.

KEEP OUT OF REACH OF CHILDREN JAUHI DARI KANAK-KANAK

PACK QUANTITIES:

Available in 60ml & 100ml PET bottle and 3.6L plastic bottle (for tender / export only).

Further information can be obtained from your pharmacist, physician or the manufacturer.

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